



Ultrasound Report – Neck Soft Tissues

Department: Diagnostic Imaging – Ultrasound
Facility: NorthStar Imaging Centre – Toronto
Exam: Ultrasound Neck (Bilateral)
Date of Exam: 2021-10-18
Comparison: None available at time of interpretation

****Indication:**** Palpable left upper anterior neck mass / lymph node. Persistent LAN without constitutional symptoms. Evaluate morphology and guide decision for sampling.

****Technique:**** High■frequency linear transducer ultrasound of bilateral neck soft tissues with targeted assessment of level II and adjacent nodal stations. Color Doppler used to assess vascularity. Measurements recorded in three dimensions where feasible.

****Findings:**** In the left level II region (upper anterior cervical chain), a dominant lymph node is identified with ****moderately thickened cortical mantle**** and preserved overall oval configuration. The hilum is partially visualized. The dominant node measures approximately ****32 × 10 × 28 mm****. A second, smaller node in the same chain demonstrates mild cortical prominence but maintains a benign-appearing shape. Right-sided level II nodes are present and mildly prominent; cortical thickness is less pronounced than the dominant left node. No discrete thyroid mass is seen on the limited field of view. There is no drainable fluid collection.

****Doppler:**** Hilar vascularity is present; there is no marked peripheral hypervascularity.

****Impression:**** (1) Dominant left level II lymph node with cortical thickening; morphology is ****atypical**** for simple reactive adenopathy but not definitively malignant. (2) Given persistence and size, ****ultrasound■guided FNA**** is recommended for cytology and flow cytometry. (3) Correlate with clinical history, laboratory markers, and any additional sites of adenopathy if present.

****Communication:**** Findings and recommendation for FNA were communicated to the ordering clinician via electronic results routing.



Addendum / Comparison and Administrative Notes

Comparison statements reflect the most recent prior studies available in the network at the time of reporting. If external imaging exists, formal comparison may be limited until images are imported. Clinicians are encouraged to interpret findings in the context of the patient's broader longitudinal record and to correlate with physical examination and symptoms.

Quality Assurance: This report was auto-formatted from a structured radiology/clinic template and reviewed for internal consistency. Minor variations in measurements may occur due to technique, probe position, and interpreter variability.